

Preventing diabetes

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Diabetes mellitus is now recognized as a growing threat to public health in almost all countries of the world. No longer considered as a “disease of affluence”, it increasingly affects Third World communities. From Pakistan to Mexico, epidemiological surveys have shown that 10% or more of the adult population now suffer from it and, worldwide, a total of 100 million prevalent cases are anticipated by the end of the century.

Insulin-dependent diabetes mellitus (IDDM), which often occurs in childhood and adolescence, requires daily injections of insulin to sustain life. But non-insulin-dependent diabetes mellitus (NIDDM), which usually occurs in middle-age or thereafter, is the most common form and comprises 85-90% of all diabetes. Both forms have severe complications – blindness, kidney failure, gangrene leading to amputation of limbs, and a high risk of cardiovascular disease.

Although the picture is alarming, there are reasons for optimism. Firstly, there is growing evidence that both IDDM and NIDDM may be prevented. A recent WHO report on diabetes prevention concluded that much of the increase in diabetes prevalence may be blamed on unhealthy lifestyles. Obesity, lack of regular physical exercise and inappropriate diet (too much refined carbohydrate and fat) lie at the root of the increase in NIDDM. These are factors that may be modified by the individual at risk or by whole communities. Several encouraging studies have suggested that IDDM too may eventually prove to be preventable.

The most costly aspect of diabetes, both to the individual and to society as a whole, is its long-term complications. Here again there is considerable scope for prevention. Much blindness due to diabetic retinopathy may be prevented today by early detection and appropriate therapy. In many developing countries, frequent foot ulcers and infections, leading to gangrene and loss of limbs as well as to systemic infection and death, may largely be eliminated by proper hygiene and adequate footwear. Thus, major improvements are possible with modest interventions and adequate patient education.

WHO is responding to this challenge at both the global and regional level, since both the WHO Regional Offices for Europe and for the Eastern Mediterranean have set goals and targets for diabetes control, including reductions in acute and long-term complications. WHO also supports a growing number of countries, in all regions, which have decided to establish national

diabetes programmes. Guidelines for the development of these programmes have been prepared by WHO Headquarters and been translated into all WHO official languages. WHO recently published the report of a Study Group on the prevention of diabetes. It was distributed to over 6000 participants at the International Diabetes Federation (IDF) Congress in Kobe, Japan, in November 1994. WHO works closely with the IDF to raise awareness of diabetes, and has co-sponsored *World Diabetes Day* on 14 November each year since 1991.

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An epidemiological survey under way in Pakistan. Worldwide, some 10% of the adult population suffer from diabetes.